EP
02

fläsh. Campus

LAYOUT 01 · COVER

In-Office Whitening Workflow

Step-by-Step Protocol + Common Mistakes



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500+ lectures · 43 countries

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LAYOUT 02 · CHAPTER

Apply

The gel is engineered for precision.

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LAYOUT 03 · HOOK

“My friend had her whitening done here — hers looks incredible. Will mine?”

THE HARD TRUTH

Same gel, same lamp — two clinics, visibly different results. The chemistry never changes between offices. The workflow does.

THE FLÄSH ANSWER

If the outcome depends on who happens to be working that day, you have a whitening lottery. A strict, standardized protocol ends the lottery.

LAYOUT 04 · CLOSING

Make It Your Team’s Reflex

1. **Run one full dry run**
   No patient in the chair — your assistant drives the fläshcard start to finish.
2. **Appoint an SOP owner**
   One team member owns this protocol — and audits the clinic against it.
3. **Laminate the phases**
   Visible chairside, next to the Seven Sins of Improper Isolation.

UP NEXT

Episode 03 · Sensitivity Prevention & Management

Preventing it, managing it — and turning a complaint into loyalty.

LAYOUT 05 · POINTS

1 mm. Green. Neutral pH.

01 Dual-chamber 32% HP.

Mix fresh — full potency protected until the moment of mixing.

02 Green = precision.

You see every surface covered — the gel turns clear as oxygen releases.

03 Oxidation, not erosion.

Oxidation of chromogens at a neutral pH — never corroding the enamel.

04 Minimum 1 mm, everywhere.

A thin, starved layer produces a pale patch — Sin #5.

LAYOUT 06 · POINTS_PHOTO

Soft Tissue First

01 Vitamin E or vaseline on the lips.

Then the fläsh lip retractor, docked onto the lamp head.

02 Filtered eye protection.

For the patient and for every team member present.

03 Gingiva protector, light-cured.

Dry gingiva; only slight overlap onto cervical enamel; cure until solid.



Lamp docked on the lip retractor · filtered goggles

EVIDENCE

Li & Greenwall, Br Dent J 2013

LAYOUT 07 · POINTS_CHART

The Lamp: Position Is Power

01 Lock the geometry.

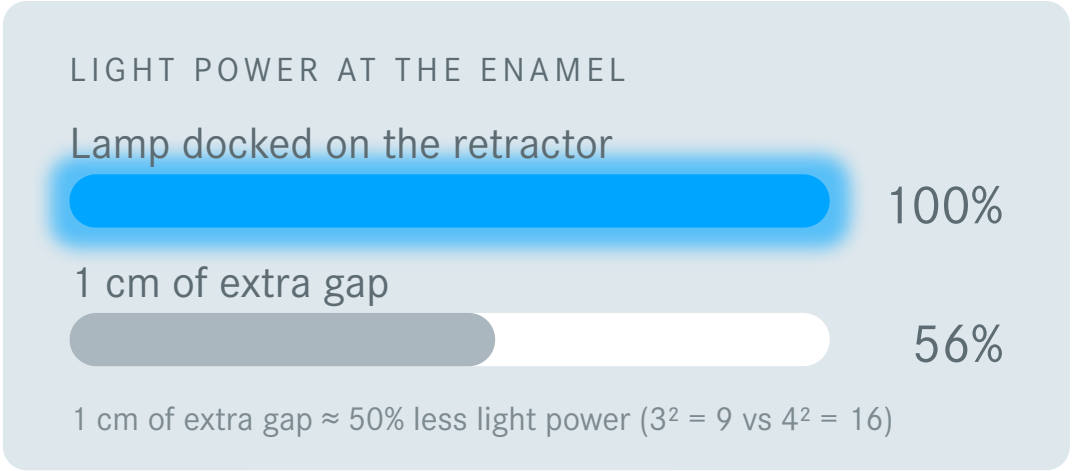
Patient at 45°, lamp head locked on the retractor — intensity level IIII.

02 The expensive centimetre.

One centimetre of gap costs ≈50% of the power.

03 Complementary-colour activation.

Blue 460 nm light, absorbed by the green gel — no UV, no infrared, below 39 °C.



EVIDENCE

Ontiveros & Paravina, J Dent 2009

LAYOUT 08 · STATEMENT

30

years of German-made whitening expertise

Gel, lamp and fläshcard engineered as one closed system · WHITEsmile

LAYOUT 09 · CHECKLIST

The Seven Sins of Isolation

1. No lip protection — vitamin E or vaseline
2. Wrong retractor for the system
3. No filtered eye protection
4. Barrier not properly placed or cured
5. Not enough whitening gel (below 1 mm)
6. Patient left unattended — no signal agreed
7. Dentin sensitivity not managed

CLINICAL FOCUS

Burns are never caused by the gel.

Rescue: vitamin E or vaseline onto the gingiva — no panic, protocol.

LAYOUT 10 · STEPS

The Fix & The Conversation

1 Re-isolate, correctly

Rebuild the barrier and confirm the retractor is docked on the lamp head.

2 Fresh 1 mm gel

Full-thickness layer on the canines and cervical thirds — the lagging zones.

3 Spend banked minutes

The cycle time the fläshcard saved becomes a targeted touch-up.

4 Say the words

“The canines are always last. The white areas are temporary.” Book the one-week photo.

LAYOUT 10 · STEPS WITH PHOTOS

Apply & Activate



Apply the gel, 1 mm thick



Adjust the lamp, start 15 min



Suction off the gel — no rinsing

Repeat 3× (max 4×) — fresh gel each cycle, suction only, never rinse.

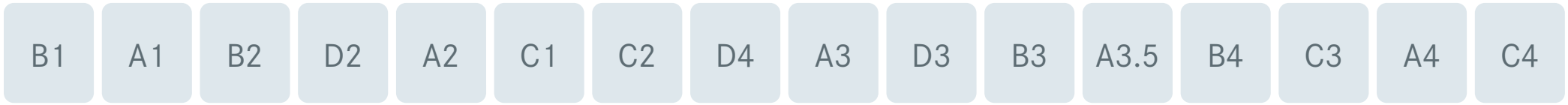
LAYOUT 11 · COMPARE

Oxidation, Not Erosion

MYTH	FACT
✕ Whitening etches the enamel	✓ Oxidation of chromogens at neutral pH
✕ The lamp works by heating the tooth	✓ Blue 460 nm light, below 39 °C — a photochemical effect
✕ Burns come from the gel	✓ Burns are never caused by the gel — isolation prevents them

LAYOUT 12 · SCALE

The Value-Ordered Baseline



Value-ordered shade guide · lightest to darkest

01 Determine it together.

Daylight, teeth still hydrated, the tab in the patient’s own hand.

02 Enter it into the lamp.

After treatment it reports the shades gained — a number, not an opinion.

EVIDENCE

Joiner A., J Dent 2004

LAYOUT 13 · SUMMARY

The SOP Blueprint

Document
Clean first, baseline shade in value order, photograph like a scientist.

Protect
Vitamin E or vaseline, correct retractor, goggles, cured barrier — avoid the Seven Sins.

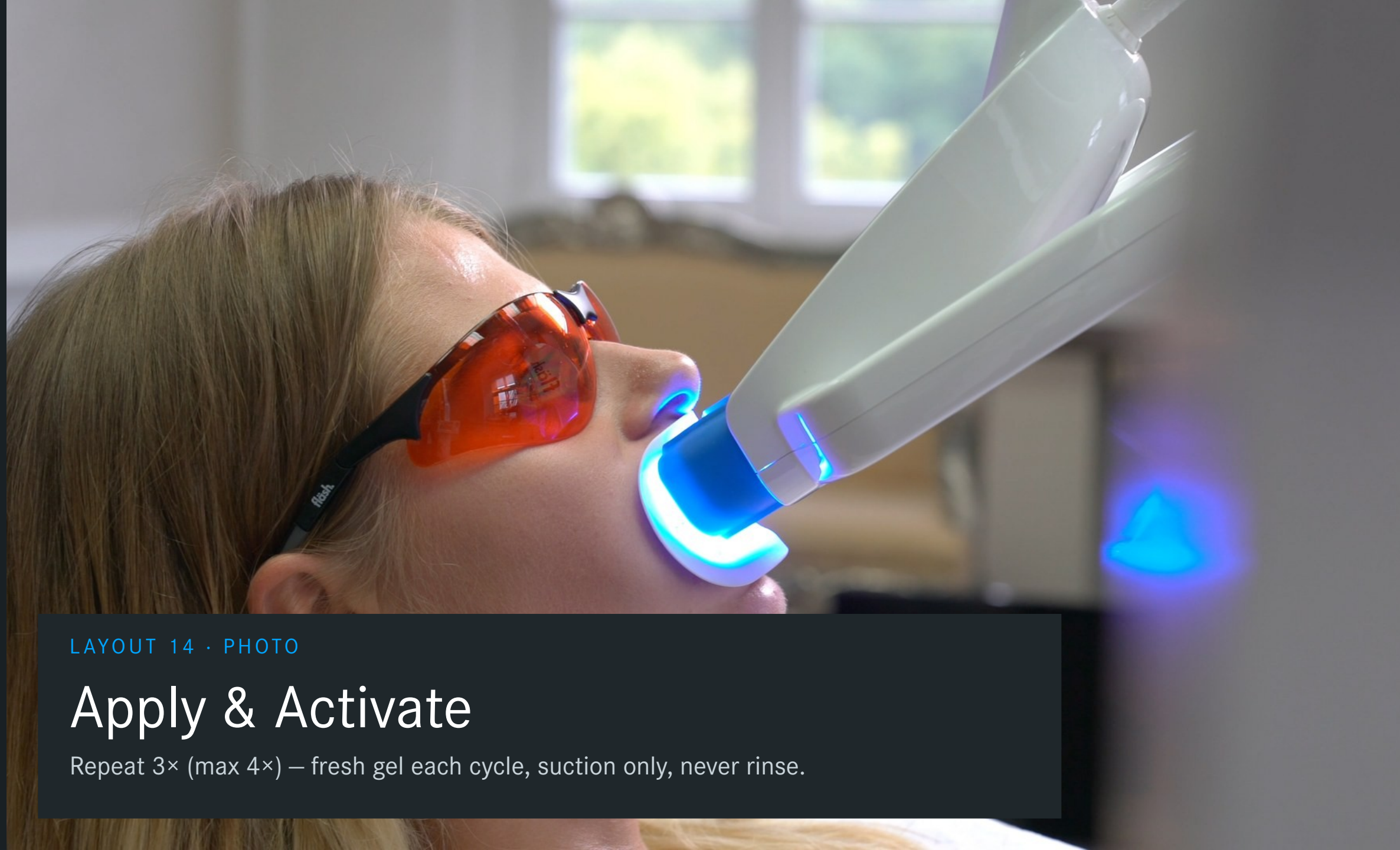
Apply
A minimum 1 mm of green gel, working by oxidation at neutral pH.

Activate & monitor
Lamp locked on the retractor · fläshcard discipline · comfort checkpoints.

Seal & instruct
Mousse, the aftercare window, the departure gift — identically, every time.

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LAYOUT 14 · PHOTO

Apply & Activate

Repeat 3× (max 4×) — fresh gel each cycle, suction only, never rinse.

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LAYOUT 15 · BEFORE_AFTER

Clean Before You Whiten



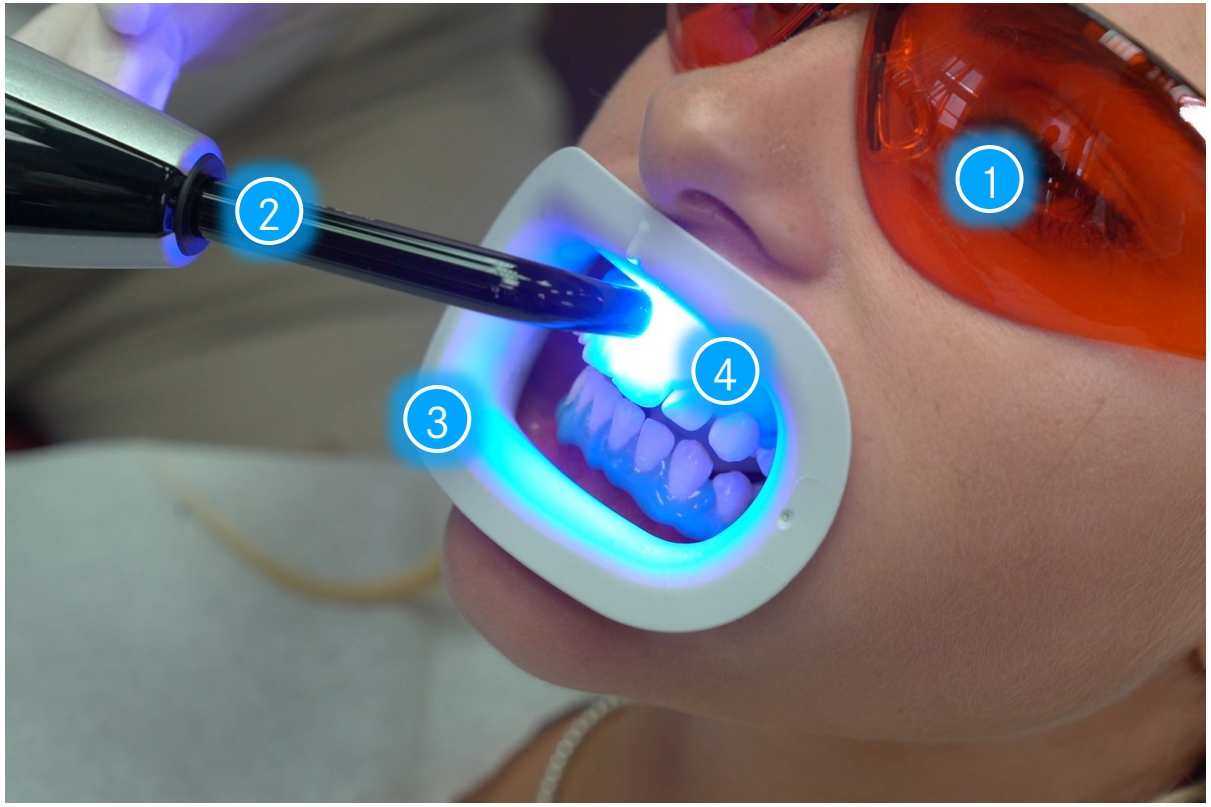
Before prophylaxis



After prophylaxis

LAYOUT 16 · PHOTO_ANNOTATED

The Isolation Setup



1. Filtered goggles
2. Curing light
3. Lip retractor
4. Gingiva Protector, light-cured

LAYOUT 17 · CASE

“The Uneven Case”

A 38-year-old at the end of cycle 3 — centrals magnificent, cervical thirds still yellow, canines two steps behind, chalky patches on the laterals. She is holding the mirror.

FINDING	CAUSE
Cervical thirds still yellow	➡ Barrier over-extended onto cervical enamel
Patchy brightness	➡ Gel layer below 1 mm in zones — Sin #5
Everything underperforming	➡ Lamp gap or tilt — 1 cm ≈ −50% power
Canines two steps behind	➡ Thicker dentin — biology, not error
Chalky white patches	➡ Transient dehydration — not damage

LAYOUT 18 · QUIZ

Cervical thirds still yellow after cycle 3 — what is the most likely cause?

A Gel layer below 1 mm

B Barrier over-extended onto cervical enamel

C Lamp gap or tilt

LAYOUT 18 · QUIZ

Cervical thirds still yellow after cycle 3 — what is the most likely cause?

A Gel layer below 1 mm

B Barrier over-extended onto cervical enamel

C Lamp gap or tilt

The barrier covered the cervical enamel — the gel never touched it. Re-isolate correctly, then apply fresh 1 mm gel.